

Executive Overview & Response Performance

Dispatcher_Name

Incident_Type

Gender

Priority_Level

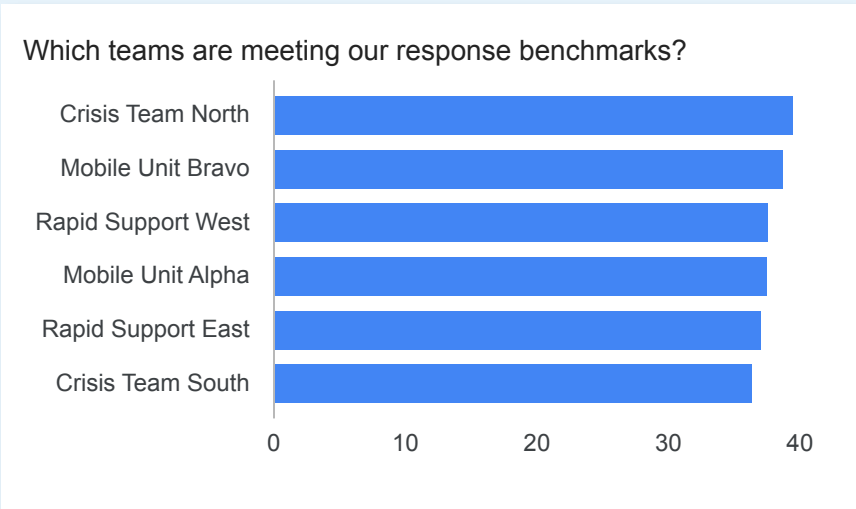
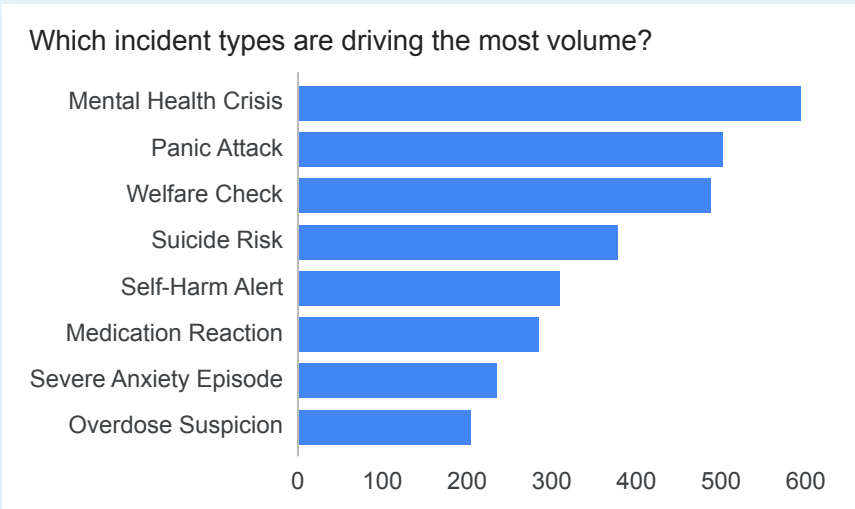
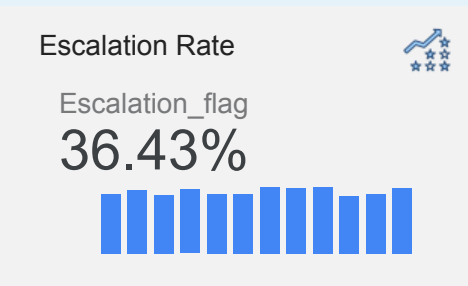
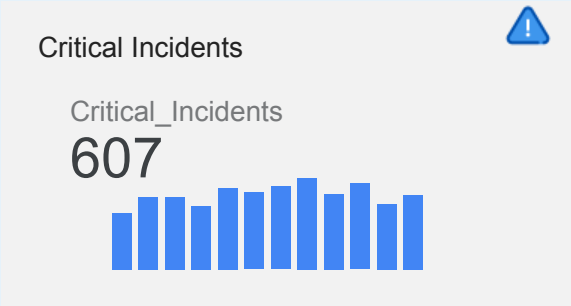
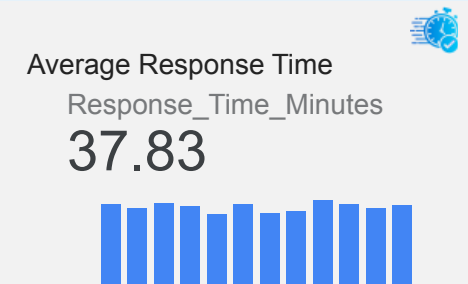
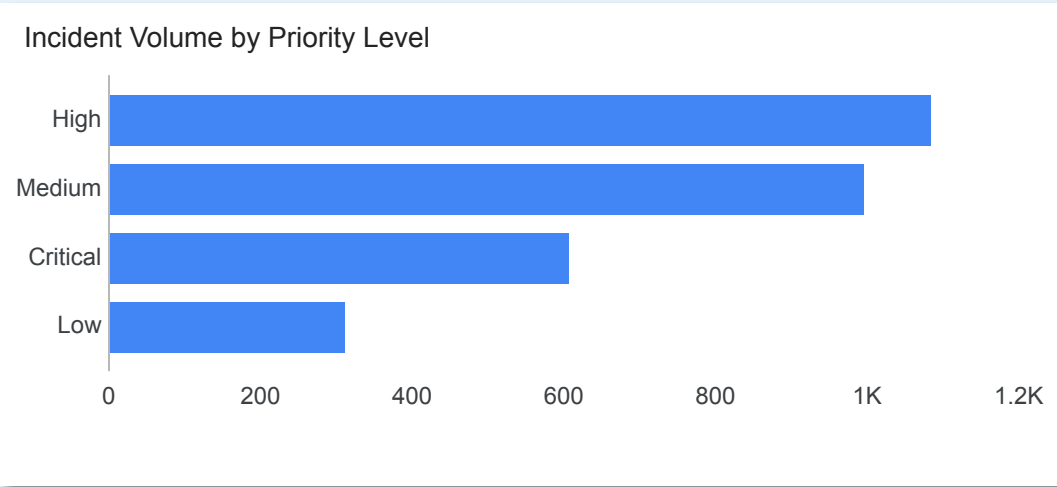
Welcome All, To the Real-Time Emergency Monitoring Dashboard

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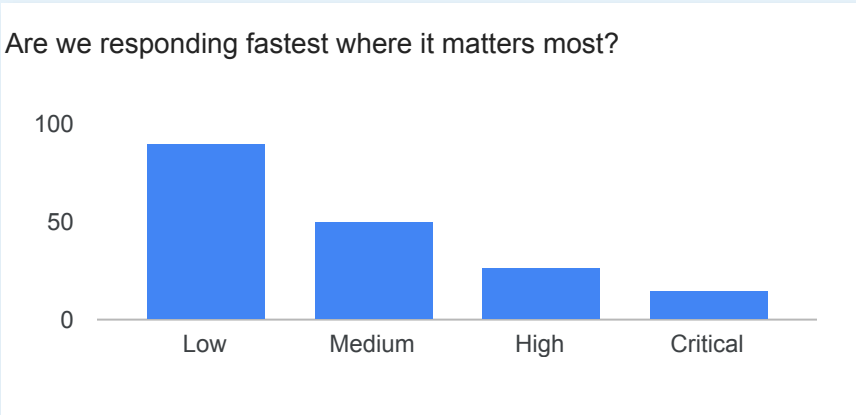
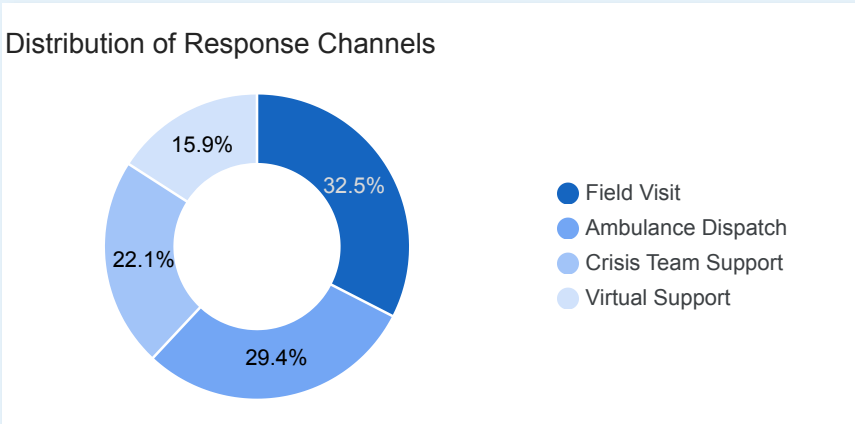
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Key insight: Response speed is inverted across priority levels. Critical incidents receive the slowest average response time, despite accounting for 607 cases out of 3,000 total and driving a 36.43% escalation rate. The cases most in need of urgent attention are consistently being reached last, suggesting current dispatch prioritisation may be misaligned with actual risk, and may be contributing directly to the high rate of incidents escalating beyond their initial classification.



Patient Outcome Insights & Geographic Analysis

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Gender

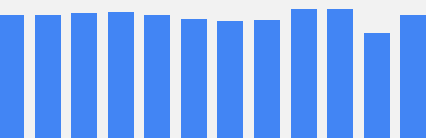
Priority_Level

Total Incidents



Record Count

3,000



Average Response Time



Response_Time_Minutes

37.83

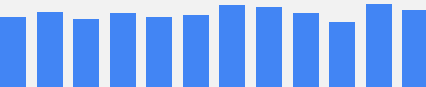


Escalation Rate



Escalation_flag

36.43%



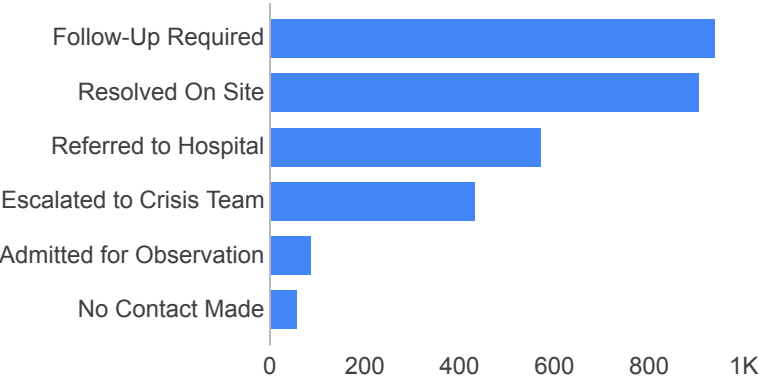
Critical_Incidents



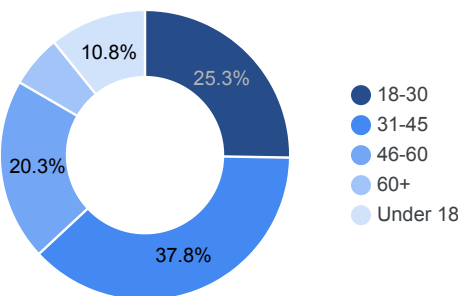
607



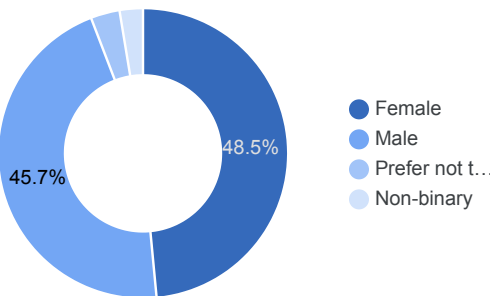
When are our peak incident periods?



Patient Age Distribution



Gender Distribution

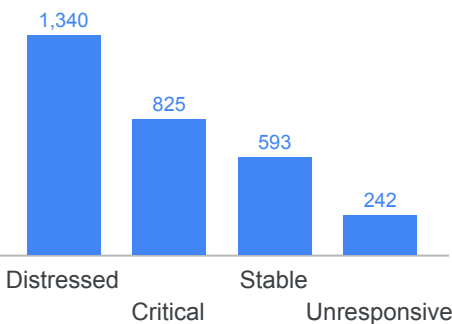


Incidents by City & Region

	City	Region	Record Count
1.	Bristol	Bristol	211
2.	Nottingham	Nottinghamshire	200
3.	Croydon	Greater London	182
4.	London	Greater London	167
5.	Bromley	Greater London	166
6.	Enfield	Greater London	165
7.	Leicester	Leicestershire	159
8.	Salford	Greater Manches...	150
9.	Sheffield	South Yorkshire	136
10.	Doncaster	South Yorkshire	134
11.	Bolton	Greater Manches...	133
12.	Wolverhampton	West Midlands	131
13.	Liverpool	Merseyside	129
14.	Manchester	Greater Manches...	127
15.	St Helens	Merseyside	123
16.	Birmingham	West Midlands	121
17.	Coventry	West Midlands	119
18.	Huddersfield	West Yorkshire	105
19.	Bradford	West Yorkshire	97
20.	Leeds	West Yorkshire	96
21.	Newcastle	Tyne and Wear	78
22.	Sunderland	Tyne and Wear	71

Key Insights : Distress, not medical criticality, dominates patient outcomes. 1,340 patients (the largest single category) were found Distressed, over 5x more than Unresponsive (242), and De-escalation Support ranks among the top-3 interventions used, reinforcing this pattern. Cases concentrate most heavily in Bristol (211) and Nottingham (200), pointing to where de-escalation-trained response capacity may be most needed.

Patient Condition at Incident



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Operational Risk & Resource Efficiency

Welcome All, To the Real-Time Emergency Monitoring Dashboard

Dispatcher_Name ▾

Incident_Type ▾

Gender ▾

Priority_Level ▾

Response_Time_Minutes

37.83



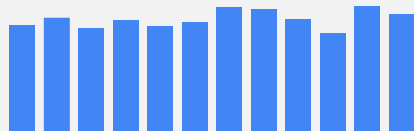
Critical_Incident_Rate

0.28%



Escalation_Rate

0.36%



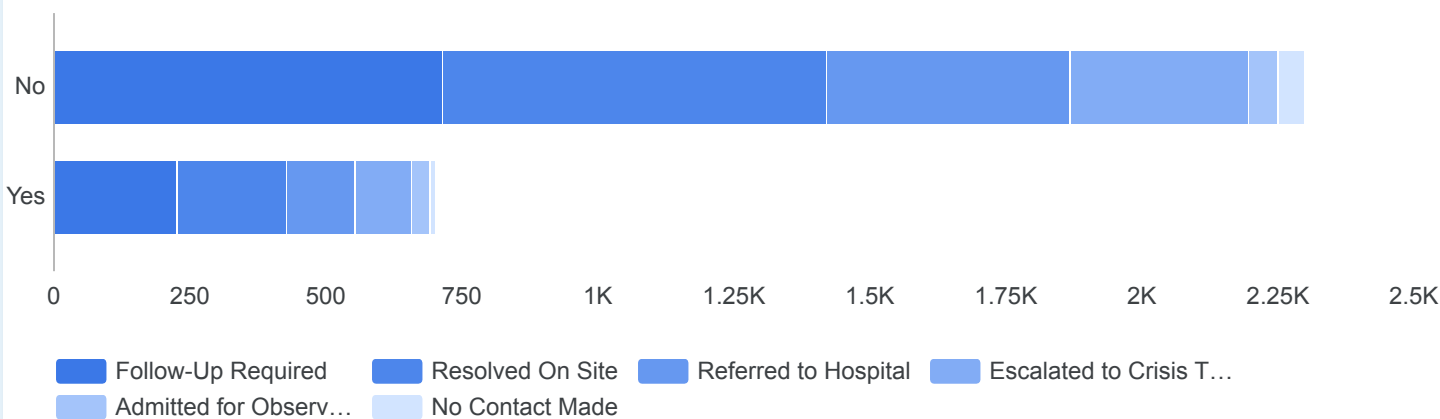
Vulnerable_Patient_Rate

0.23%

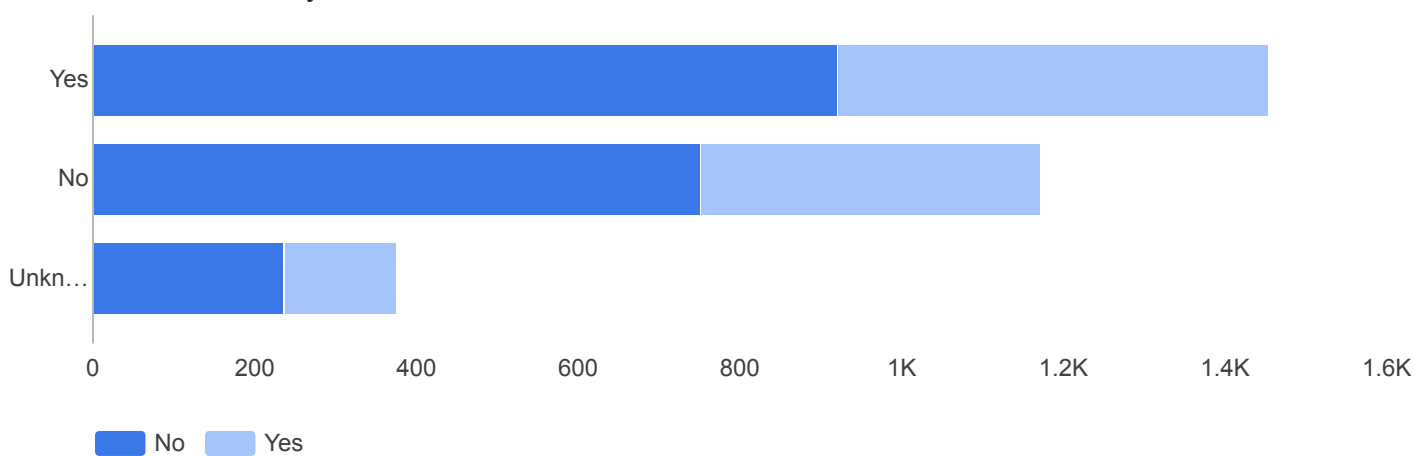


Key Insights: Patients with known mental health history drive disproportionately high escalation, yet Virtual Support, their most likely response channel, has the slowest average response time of all modes, while Ambulance Dispatch responds fastest despite lower relative urgency in this context.

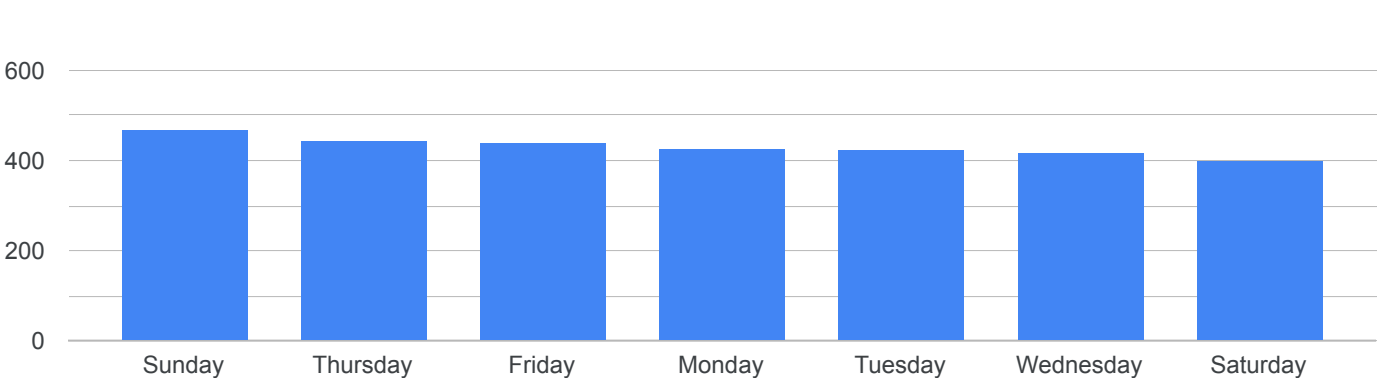
Incident Outcomes Among Vulnerable Patients



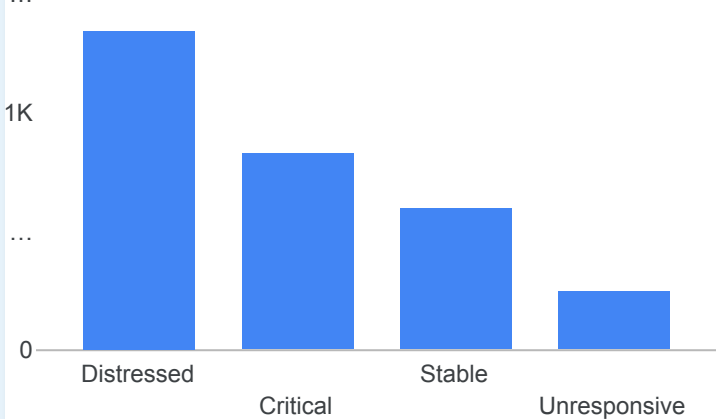
Mental Health History vs Escalation Outcomes



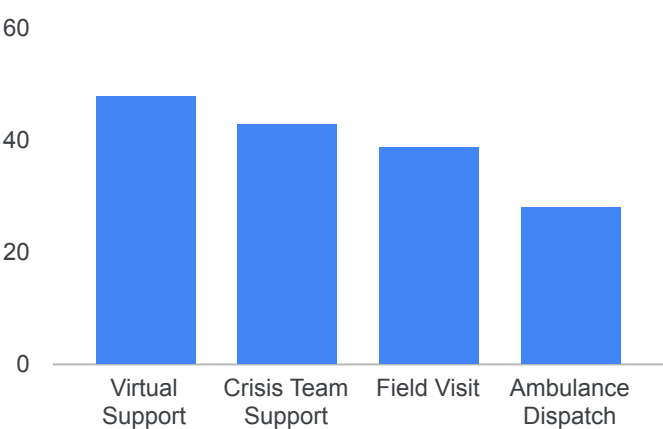
Incident Volume by Day of Week



Patient Condition at First Contact



Average Response Time by Response Mode



Response Time vs Incident Outcomes

